

## FRONT

1 cm  
Barcode**hovid** Ciprofloxacin Eye Drops 0.3% w/v

VICIP01-PC

**DESCRIPTION**

Clear, colourless liquid.

**COMPOSITION**

Each 8 mL contains Ciprofloxacin Lactate equivalent to Ciprofloxacin 24 mg.

**ACTIONS AND PHARMACOLOGY**

Ciprofloxacin eye drops contains the fluoroquinolone ciprofloxacin. The cidal and inhibitory activity of ciprofloxacin against bacteria results from an interference with the DNA gyrase, an enzyme needed by the bacterium for the synthesis of DNA. Thus the vital information from the bacterial chromosomes cannot be transcribed which causes a breakdown of the bacterial metabolism. Ciprofloxacin has *in vitro* activity against a wide range of Gram-positive and Gram-negative bacteria.

**Commonly susceptible species**

*Gram-positive aerobes:* *Corynebacterium acroclens*, *Corynebacterium auris*, *Corynebacterium propinquum*, *Corynebacterium pseudodiphthericum*, *Corynebacterium striatum*, *Staphylococcus aureus* (methicillin susceptible-MSSA), *Staphylococcus capitis*, *Staphylococcus epidermidis* (methicillin susceptible-MSE), *Staphylococcus hominis*, *Staphylococcus saprophyticus*, *Staphylococcus warneri*, *Streptococcus pneumoniae*, *Streptococcus viridans* Group

*Gram-negative aerobes:* *Acinetobacter species*, *Haemophilus influenzae*, *Moraxella catarrhalis*, *Pseudomonas aeruginosa*, *Serratia marcescens*

**Species for which acquired resistance may be a problem**

*Gram-positive aerobes:* *Staphylococcus aureus* (methicillin resistant-MRSA), *Staphylococcus epidermidis* (methicillin resistant-MRSE), *Staphylococcus lugdunensis*

**Inherently resistant organisms**

*Gram-positive aerobes:* *Corynebacterium jeikeium*

**PHARMACOKINETICS**

hovid Ciprofloxacin Eye Drops is rapidly absorbed into the eye following topical ocular administration. Systemic levels are low following topical administration. Plasma levels of ciprofloxacin in human subjects following 2 drops of 0.3% ciprofloxacin solution every 2 hours for two days and then every four hours for 5 days ranged from non-quantifiable (<1.0 ng/mL) to 4.7 ng/mL. The mean peak ciprofloxacin plasma level obtained is approximately 450-fold less than that seen following a single oral dose of 250 mg ciprofloxacin. The systemic pharmacokinetic properties of ciprofloxacin have been well studied. Ciprofloxacin widely distributes to tissues of the body. The apparent volume of distribution at steady state is 1.7 to 5.0 l/kg. Serum protein binding is 20-40%. The half-life of ciprofloxacin in serum is 3-5 hours. Both ciprofloxacin and its four primary metabolites are excreted in urine and faeces. Renal clearance accounts for approximately two-thirds of the total serum clearance with biliary and faecal routes accounting for the remaining percentages. In patients with impaired renal function, the elimination half-life of ciprofloxacin is only moderately increased due to extrarenal routes of elimination. Similarly, in patients with severely

reduced liver function the elimination half-life is only slightly longer.

**INDICATION**

hovid Ciprofloxacin Eye Drops is indicated for the treatment of corneal ulcers and superficial infections of the eye and adnexa caused by susceptible strains of bacteria.

**CONTRAINDICATIONS**

- Hypersensitivity to ciprofloxacin hydrochloride
- Hypersensitivity to quinolones

**WARNINGS AND PRECAUTIONS**

- After cap is removed, if tamper evident snap collar is loose, remove before using product.
- The clinical experience in children less than one year old, particularly in neonates is very limited. The use of ciprofloxacin eye drops in neonates with ophthalmia neonatorum of gonococcal or chlamydial origin is not recommended as it has not been evaluated in such patients.
- When using ciprofloxacin eye drops one should take into account the risk of rhinopharyngeal passage which can contribute to the occurrence and the diffusion of bacterial resistance.
- Serious and occasionally fatal hypersensitivity (anaphylactic) reactions, some following the first dose, were observed in patients receiving treatment based on systematically administered quinolones. Some reactions were accompanied by cardiovascular collapse, loss of consciousness, tingling, pharyngeal or facial oedema, dyspnoea, urticaria and itching. Only a few patients had a history of hypersensitivity reactions.
- Serious acute hypersensitivity reactions to ciprofloxacin may require immediate emergency treatment. Oxygen and airway management should be administered where clinically indicated.
- Discontinue use at the first appearance of skin rash or any other sign of hypersensitivity.
- As with all antibacterial preparations prolonged use may lead to overgrowth of non-susceptible bacterial strains or fungi. If superinfection occurs, appropriate therapy should be initiated.
- Tendon inflammation and rupture may occur with systemic fluoroquinolone therapy including ciprofloxacin, particularly in elderly patients and those treated concurrently with corticosteroids. Therefore, treatment with ciprofloxacin eye drops should be discontinued at the first sign of tendon inflammation.
- In patients with corneal ulcer and frequent administration of ciprofloxacin eye drops, white topical ocular precipitates (medication residue) have been observed which resolved after continued application of ciprofloxacin eye drops. The precipitate does not preclude the continued application of ciprofloxacin eye drops nor does it adversely affect the clinical course of the recovery process. The onset of the precipitate was within 24 hours to 7 days after starting therapy. Resolution of the precipitate varied from immediately to 13 days after therapy commencing.

hovid

BACK

1 cm  
Barcode

**hovid Ciprofloxacin Eye Drops 0.3% w/v**

**Gastrointestinal disorders**

*Common:* Dysgeusia

*Uncommon:* Nausea

*Rare:* Diarrhoea, abdominal pain

**Skin and subcutaneous tissue disorders**

*Rare:* Dermatitis

**Musculoskeletal and connective tissue disorders**

*Not known:* Tendon disorder

**OVERDOSE AND TREATMENT**

A topical overdose may be rinsed out from the eye(s) with lukewarm tap water. Due to the characteristics of this preparation, no toxic effects are to be expected with an ocular overdose of this product, or in the event of accidental ingestion of the contents of one bottle.

**DOSAGE AND ADMINISTRATION**

Adults, newborn infants (0-27 days), infants and toddlers (28 days to 23 months), children (2-11 years) and adolescents (12-16 years)

**Corneal Ulcers**

*Ciprofloxacin eye drops must be administered in the following intervals, even during night time:*

On the first day, instil 2 drops into the affected eye every 15 minutes for the first six hours and then 2 drops into the affected eye every 30 minutes for the remainder of the day.

On the second day, instil 2 drops in the affected eye hourly.

On the third through the fourteenth day, place two drops in the affected eye every 4 hours. If the patient needs to be treated longer than 14 days, the dosing regimen is at the discretion of the attending physician.

**Superficial Ocular Infection**

The usual dose is one or two drops in the affected eye(s) four times a day. In severe infections, the dosage for the first two days may be one or two drops every two hours during waking hours.

For either indication a maximum duration of therapy of 21 days is recommended.

The dosage in children above the age of 1 year is the same as for adults.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

**Storage:**

Store below 30°C. Protect from light.

It is recommended to discard remaining contents one month after opening.

**Presentation/ Packing:**

Eye Drops of 8 mL, 1 bottle per box.

Product Registration Holder / Repacked by:

HOVID Bhd.,

121, Jalan Tunku Abdul Rahman,

30010 Ipoh, Malaysia.

Manufactured by: Jiangsu Farover Pharmaceutical Co., Ltd.

No. 18, Yangshan Road Street Avenue,

Economic & Development Zone, Xuzhou City,

Jiangsu Province, China.

Information date: May 2016

hovid

- Contact lens wear is not recommended during treatment of an ocular infection. Therefore, patients should be advised not to wear contact lenses during treatment with ciprofloxacin eye drops.
- Avoid contact with soft contact lenses. In case patients are allowed to wear contact lenses they should be instructed to remove them prior to application of ciprofloxacin eye drops and wait at least 15 minutes before reinsertion.

**PREGNANCY AND LACTATION**

**Pregnancy:** There are no adequate data from the use of ciprofloxacin in pregnant woman. As a precautionary measure, it is preferable to avoid the use of ciprofloxacin during pregnancy, unless the therapeutic benefit is expected to outweigh the potential risk to the fetus.

**Lactation:** It is unknown whether ciprofloxacin is excreted in human breast milk following topical ocular or otic administration. A risk to the suckling child cannot be excluded. Therefore, caution should be exercised when ciprofloxacin is administered to nursing women.

**DRUG INTERACTIONS**

Given the low systemic concentration of ciprofloxacin following topical ocular administration of the product, drug interactions are unlikely to occur.

If more than one topical ophthalmic medicinal product is being used, the medicines must be administered at least 5 minutes apart. Eye ointments should be administered last.

**MAIN SIDE/ ADVERSE EFFECTS**

The adverse reactions listed below are classified according to the following convention: very common ( $\geq 1/10$ ), common ( $\geq 1/100$  to  $<1/10$ ), uncommon ( $\geq 1/1,000$  to  $<1/100$ ), rare ( $\geq 1/10,000$  to  $<1/1,000$ ), very rare ( $<1/10,000$ ), or not known (cannot be estimated from the available data). Within each frequency-grouping, adverse reactions are presented in order of decreasing seriousness.

**Immune system disorders**

*Rare:* Hypersensitivity

**Nervous system disorders**

*Uncommon:* Headache

*Rare:* Dizziness

**Eye disorders**

*Common:* Corneal deposits, ocular discomfort, ocular hyperaemia

*Uncommon:* Keratopathy, punctate keratitis, corneal infiltrates, photophobia, visual acuity reduced, eyelid oedema, blurred vision, eye pain, dry eye, eye swelling, eye pruritus, lacrimation increased, eye discharge, eyelid margin crusting, eyelid exfoliation, conjunctival oedema, erythema of eyelid

*Rare:* Ocular toxicity, keratitis, conjunctivitis, corneal epithelium defect, diplopia, hypoaesthesia eye, asthenopia, eye irritation, eye inflammation, hordeolum

**Ear and labyrinth disorders**

*Rare:* Ear pain

**Respiratory, thoracic and mediastinal disorders**

*Rare:* Paranasal sinus hypersecretion, rhinitis